



Situation Report of the 17th Ebola Virus Disease Outbreak / DRC

SitRep N°091/MVEBDB/08/13/2026

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**HIGHLIGHTS** (last 24 hours)

In the last 24 hours, 62 new confirmed cases and 30 deaths (14 community deaths and 16 intra-Ebola Virus Disease (EVD) cases) have been recorded in the provinces of Ituri (41 cases), North Kivu (16 cases), Haut Uélé (4) and Tshopo (1). A new health zone affected (Tshopo) in the province of Tshopo.

ACCUMULATIVE CASE 4,727	ACCUMULATIVE CONFIRMED DEATHS 2,214	LETHALITY 46.8%	PATIENTS IN ISOLATION / CTE 749	ACCUMULATIVE HEALED 976	TRACKING RATES CONTACTS (Of the Day) 82.6%
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Affected Provinces and Affected Health Zones

**6 Provinces Affected**

Haut-Uélé, Ituri, North Kivu,
South Kivu, Tshopo & Bas Uélé

**55 Health Zones**

Affected Out of 151

(36.4%)

Summary of key points from the last 24 hours

In the last 24 hours, 62 new confirmed cases and 30 deaths (14 Community deaths and 16 Ebola Virus Disease (EVD) deaths within Ebola Treatment Centers (ETCs) have been reported in the provinces of **Ituri (41 cases)**, **North Kivu (16 cases)**, **Haut-Uélé (4)**, and **Tshopo (1)**. Ituri accounts for approximately two-thirds of the new cases.

From the start of the epidemic until August 13, 2026, the Democratic Republic of Congo recorded 4,727 confirmed cases, including 2,214 deaths, representing a case fatality rate of 46.8%. Ituri remains the epicenter of the epidemic, accounting for 85.0% of cumulative confirmed cases and 66.1% of new cases reported in the last 24 hours.

In Ituri, 11 patients were declared recovered after receiving two negative results for Ebola Virus Disease (EVD). However, 16 confirmed deaths within Ebola Treatment Centers (ETCs) were recorded, including 14 in Ituri and 2 in Haut-Uélé.

Thus, 55 of the 151 health zones in the six provinces have been affected since the start of the epidemic in the DRC. Ituri has 28 health zones affected out of 36, followed by North Kivu (12/34), Haut-Uélé (6/13), Tshopo (7/23), South Kivu (1/34) and Bas-Uélé (1/11).

Distribution of confirmed cases and deaths by affected province

(As of August 13, 2026)

Province	Confirmed cases	Deaths (confirmed)	Lethality	Affected health zones	New confirmed cases (24h)
Ituri	4020	1,745	43.4%	28/36 (77.8%)	41
North Kivu	569	404	71.0%	12/34 (35.3%)	16
Haut-Uélé	123	57	46.3%	6/13 (46.1%)	4
Tshopo	11	6	54.5%	7/23 (30.4%)	1
South Kivu	3	1	33.3%	1/34 (2.9%)	0
Bas Uélé	1	1	100.0%	1/11 (9.1%)	0
Total	4,727	2,214	46.8%	55/151 (36.4%)	62

Confirmed cases and deaths by province and health zone (situation as of August 13, 2026)

Province / Health Zone	Cumulative number			Current situation (24 hours)			
	Case (n)	Deaths (n)	Lethality	New Case	Community deaths (Swab+)	Confirmed deaths within the CTE	Total confirmed deaths
Ituri	4020	1745	43.4%	41	5	14	19
Adja	11	1	9.1%				0
Aru	7	6	85.7%				0
Ariwara	7	2	28.6%				0
Aungba	10	5	50.0%				0
Bamboo	104	22	21.2%	11	0		0
Boga	1	1	100.0%				0
Bunia	1100	333	30.3%	5	0		0
Damascus	21	9	42.9%				0
Drodro	13	6	46.2%				0
Fataki	62	30	48.4%	3	0		0
Gethy	2	1	50.0%				0
Kambala	2	0	0.0%				0
Kilo	31	12	38.7%	1	0		0
Komanda	43	29	67.4%	1	0		0
Lita	169	97	57.4%				0
Logo	11	5	45.5%				0
Lolwa	14	4	28.6%				0
Mahagi	4	1	25.0%				0

Mambasa	13	6	46.2%				0
Mandima	22	13	59.1%	3	0		0
Mangala	122	76	62.3%				0
Mongbwalu	581	282	48.5%				0
Nia-Nia	156	92	59.0%	5	4		4
Nizi	513	231	45.0%	4	0		0
Nyankunde	120	35	29.2%	2	0		0
Rimba	9	4	44.4%				0
Rwampara	823	319	38.8%	5	0		0
Tchomia	49	25	51.0%	1	1		1
To ventilate	N / A	98	N / A	N / A	N / A	14	14
North Kivu	569	404	71.0%	16	9	0	9
Blessed	86	65	75.6%	1	0		0
Butembo	108	91	84.3%	1	0		0
Goma	1	0	0.0%				0
Kalunguta	11	5	45.5%	1	1		1
Katwa	274	188	68.6%	12	8		8
Kyondo	14	12	85.7%				0
Lubero	1	1	100.0%				0
Mabalako	2	1	50.0%				0
Masereka	7	4	57.1%				0
Musienene	56	32	57.1%	1	0		0
Oicha	5	4	80.0%				0
Vuhovi	4	1	25.0%				0
Upper Uele	123	57	46.3%	4	0	2	2
Boma Mangbetu	22	6	27.3%			1	1
Gombari	1	1	100.0%				0
Isiro	39	19	48.7%	2	0		0
Pawa	19	14	73.7%	2	0		0
Rungu	1	0	0.0%				0
Wamba	41	17	41.5%			1	1
Tshopo	11	6	54.5%	1	0	0	0
Bafwasende	1	0	0.0%				0
Kabondo	3	2	66.7%				0
Lubunga	1	0	0.0%				0
Makiso - Kisangani	3	3	100.0%				0
Mangobo	1	1	100.0%				0
Tshopo	1	0	0.0%	1	0		0
Wanie -Rukula	1	0	0.0%				0
South Kivu	3	1	33.3%	0	0	0	0
Miti-Murhesa	3	1	33.3%	0	0	0	0

Bas Uélé	1	1	100.0%	0	0	0	0
Buta	1	1	100.0%	0	0	0	0
Total	4727	2214	46.8%	62	14	16	30

*These are the confirmed deaths that occurred in the various CTEs in the province and are being broken down to be classified by health zone.

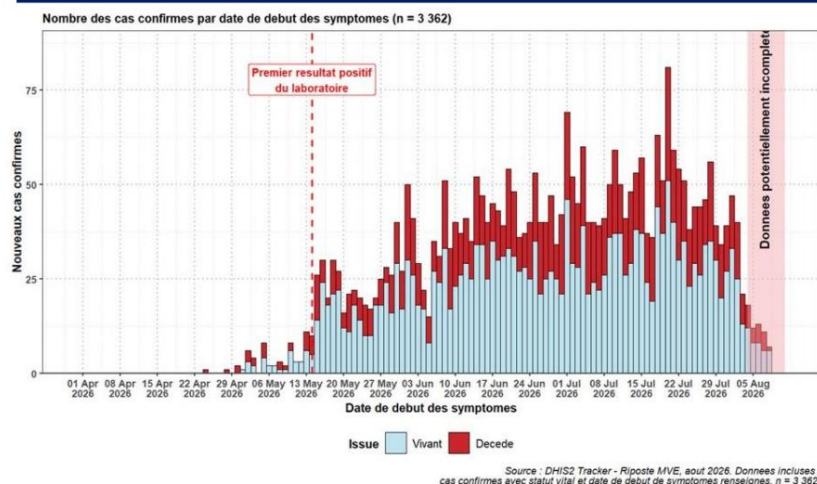
NA: Not Applicable

Status of alerts notified by province, as of August 13, 2026

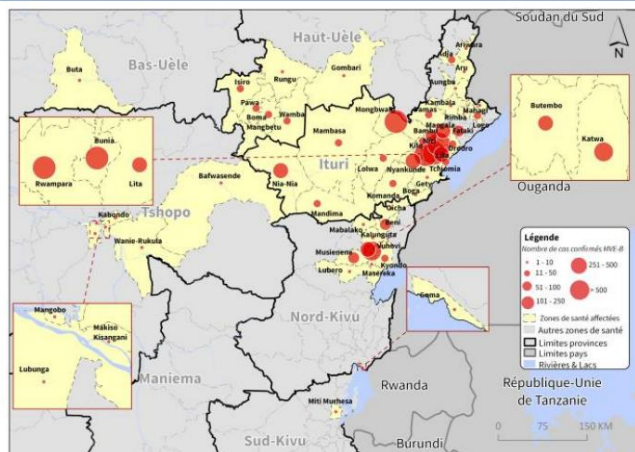
DPS	New Alerts Received		Total alerts	Alerts verified and validated (suspected case)		Alerts verified and invalidated (non-suspected cases)		Number of suspects investigated	Number of suspects investigated and transferred to CT/CTE (of the day)
	Living	Deceased		Living	Deceased	Living	Deceased		
Bas-Uele	ND	ND	ND	ND	ND	ND	ND	ND	ND
Haut-Uélé	5	0	5	5	0	0	0	5	5
Ituri	571	52	623	121	52	410	0	157	146
North Kivu	995	48	1043	103	48	883	0	149	101
South Kivu	8	0	8	1	0	7	0	1	1
Tshopo	ND	ND	0	ND	ND	ND	ND	ND	ND
Total General	1579	100	1,679	230	100	1300	0	312	253

"Alert data for Bas-Uélé is not available for the period in question."

Evolution of cases by date of symptom onset over the last 21 days



MAPPING OF CASES over the last 21 days



1. RESPONSE ACTIONS BY PILLAR

1.1. Coordination

1.1.1. Main Actions

- **North Kivu** : Support for response activities.
- **Ituri**: plea from the Chief of the Ndo Okebo chiefdom to the health authorities for the improvement of the care of patients in the Mangala health zone (Djugu territory).

1.2. Epidemiological surveillance

1.2.1. Main Actions

- By the end of the day on August 13, 2026, 1,679 alerts had been recorded, including 1,630 (97.1%) were verified in the 5 reporting provinces. Thus, 330 (20.2%) were validated as suspected cases and 312 (94.5%) were investigated, among which 253 Suspects were transferred to CT/CTE.
- The proportion of contacts followed up in the last 24 hours is at 82.6% (18,113 seen out of 21,926 to follow up), close to the threshold of 85%, with lower performance in Haut-Uélé (69.7%) which remained static.

1.2.2. Challenges

Low reporting of alerts and listing and monitoring of contacts around confirmed cases.

1.3. Update on checkpoint and point of entry (PoC/PoE) activities

1.3.1. Main actions

- Nine (9) alerts were detected at the different PoE/PoCs: four in Ituri: One live alert notified and validated at the PoC of Mud'zibala (AS Nzere, ZS of Bunia) then referred to the HGR of Bunia, one at the PoE of Kasenyi (ZS of Tchomia) referred to the HGR of Tchomia, one at the PoE of Bafwanekengele (ZS of Nia-Nia) being referred, as well as a lifeless body intercepted at the PoE of Tchomia (AS Kikoga) and five in Haut-Uélé, all alive, at the PoC of PK10 Poko road (1), at the PoC of Magambe (1) and at the PoC of Gossamu (3), in the ZS of Isiro.
- A total of 267,847 travelers were screened in the four provinces reporting their PoE/PoC data (138,816 in Ituri, 80,756 in North Kivu, 43,743 in Haut-Uélé, and 4,532 in **Tshopo**), with a screening rate of 97.5%; South Kivu also reported 84,332 people screened. In Ituri, only one of the 11 strategic PoE/PoC sites (9.1%) operated without interruption, but the overall completion rate reached 60/60 (100%); in North Kivu, all 18 sites reported (100%), and 97.35% of travelers were screened.
- In **Haut-Uélé**, 10 PoE/PoCs are activated out of the 38 priority sites (26.3%), 9 out of 10 sites have transmitted their data (90%) and 43,278 out of 43,743 travelers have been screened (99%); the Aungba PoC alone concentrates 26,308 travelers (60%) and the Salisa PoC was activated today.
- In Tshopo, 7 out of 20 PoCs are activated (35%) for a completeness of 57.1% and 99.3% of travelers screened, the data transmitted however bears the date of August 12, 2026.

1.3.2. Challenges

- Insufficient operationalization of PoE/PoC: functionality remains low in Tshopo, with only 7 out of 20 (35%) activated, and reporting completeness is limited to 51 %; in Ituri, only one strategic site out of 11 is working without interruption and screening remains low at the PoC of Pont-Shari due to a lack of involvement from men in uniform.
- Constraints affecting PoE/PoC functionality: 2,139 screening refusals (2.64%) were recorded in North Kivu, 465 (1%) in Haut-Uélé — mainly in Aungba (321) and

Magambe (95) — and 4 (0.08%) in Tshopo, motivated by denial of the disease, fear of contamination by water and fear of being interned at the CTE; there are also a break in inputs, PPE and water in all the activated PoE/PoCs of Haut-Uélé and the non-payment of service providers.

1.4. Laboratory

1.4.1. Main actions

- Ituri recorded 57 positive results (284 samples analyzed), including 41 new cases (36 live and 5 dead) and 16 new samples, while North Kivu confirmed 16 cases, including 9 deaths, Haut-Uélé 4 confirmed cases with no deaths, and Tshopo 1 positive case in the Tshopo health zone. In total, 62 new cases of Ebola virus disease were confirmed across the four provinces; South Kivu recorded no positive results from the 4 samples received.

1.5. Infection Prevention and Control (IPC/EDS)

1.5.1. Main actions

- In Ituri, 19 rings were opened out of 163 (96 reported the previous day and 67 cases retained by the pillar); 73 sites were identified and 59 decontaminated (80.8%), including 55 households and 4 healthcare facilities, leaving 14 gaps beyond 48 hours. The EDS teams recorded 77 death alerts out of 50 EDSs carried out (65%) and 40 bodies collected by swab.
- In North Kivu, 30 infection control rings were planned, 2 out of 30 households with confirmed cases were decontaminated in Beni, and 13 facilities housing cases were decontaminated (7 in Beni, 3 in Butembo, 2 in Katwa, and 1 in Musienene); 35 death alerts were recorded, including 8 in the community, 26 in healthcare facilities, and 1 at the Ebola Treatment Center (ETC). Reporting completeness remains limited to 6 out of 12 health zones (50%).
- In Tshopo, IPC activities continued around the 11th confirmed case in the health zone of Tshopo.
- In Haut-Uélé, four death alerts were recorded (two in healthcare facilities and two at the Ebola Treatment Center, including three in Wamba and one in Isiro) for four complete blood counts (100%) and four bodies collected by swab (100%); four of the four planned infection control rings were opened, and decontamination was carried out in healthcare facilities in Pawa, Wamba, Isiro, and Rungu, while household decontamination remains ongoing. Three infection prevention and control (IPC) assessments were conducted, with the Abiangama Health Center (Pawa Health Zone) achieving only 30% on the scorecard.

1.5.2. Challenges

- Weak implementation of IPC standards: limited ring opening in Ituri (19 out of 163) and partial decontamination (59 out of 73 sites), partly due to a strike by decontamination workers and insecurity in some villages; limited reporting completeness in North Kivu (6/12 health zones) and data not available in Boma Mangbetu; resistance to EDS reported in Rwampara, Nia-Nia and Tchomia; four health facilities in Haut-Uélé have an IPC score of less than 50%.

1.6. Continuity of care

1.6.1. Main actions

- In Ituri, 29 ambulances are available out of 32 allocated to the response: 58 trips successfully completed (100%). The Ebola Treatment Centers (ETCs) recorded 108 admissions and 11 recoveries in the last 24 hours; occupancy reaches 528 patients for 985 beds (53.6%), i.e. 245 confirmed cases out of 489 beds (50.1%) and 283 suspected cases out of 496 beds (57.0%), with the ETCs in Bambu and Nizi at full capacity for confirmed cases and in Aru, Kilo and Kigonze at full capacity for suspected cases.
- In North Kivu, 48 new admissions were recorded (2 confirmed at the Katwa Ebola Treatment Center and 46 suspected cases), with 34 discharges, including 31 non-cases, no recoveries, and 3 deaths among the suspected cases.

(1 in Kyondo and 2 in Masereka). Thus, 151 patients are in hospital (25 confirmed and 126 suspected) for 206 beds (73.3%), including 18 children under 5 years old.

- **In Haut-Uélé**, 4 new admissions were recorded (2 confirmed in Isiro and 2 suspected cases) and no recovered patients have been discharged; 56 patients are hospitalized (33 confirmed and 23 suspects) for 84 beds set up (46 for confirmed cases and 38 for suspected cases), exceeding the capacity of the Wamba transitional isolation center. Two confirmed deaths occurred in Ebola treatment centers (1 in Boma Mangbetu and 1 in Wamba).
- **In Tshopo**, 6 new admissions (1 confirmed and 5 suspected) bring to 8 the number of patients in isolation on 12 beds (66.7%), 20 people having benefited from the experimental treatment in cumulatively, of which 11 are in progress.
- In South Kivu, 6 suspected cases are being monitored in isolation at the Katana health zone.

1.6.2. Challenges

Insufficient availability of referral resources (3 ambulances out of service in Ituri, need for 10 additional ambulances in Haut-Uélé) and low capacity of some CTE/CT: the CTEs of Bambu (186% occupancy for confirmed cases) and Nizi (200%) are saturated in Ituri and the isolation center of Wamba exceeds its capacity; Haut-Uélé also reports the absence of standardized CTEs in its six affected health zones and stock shortages of PPE are reported in North Kivu.

1.7. Risk communication and community engagement

1.7.1. Main actions

- **In Ituri**, 387 contacts were listed around 16 cases in eleven health areas; 1,017 people were reached during home visits, 300 during educational talks, 132 during community dialogues and 9,299 during mass awareness campaigns, with 22 radio programs produced and 36 community radio stations involved.
- In North Kivu, 200 motorcycle taxi parking managers were engaged in compliance with prevention measures; a guided tour of the Katwa CTE was organized for CREC pillar supervisors and 21 rumors were clarified (6 in Beni, 4 in Katwa, 3 in Masereka, 2 in Lubero, 2 in Oicha and 2 in Vuhovi), with 11 community incidents remaining pending resolution in Musienene.
- In Tshopo, communication activities continued in the health zones of Makiso-Kisangani and Tshopo surrounding the 11th confirmed case. In Haut-Uélé, 313 people were reached during mass awareness campaigns (102 motorcycle taxi drivers and 211 people at the Duembe market, Mayogo health zone), 5 out of 14 radio stations broadcast the messages, and 41 IEC (Information, Education, and Communication) tools were distributed, bringing the total to 340.

1.7.2. Challenges

Persistent community challenges: refusal of dignified and safe burial teams in Ituri (Rwampara, Nia-Nia, Tchomia) and in Komanda, Mambasa, Aru and Mandima; 11 unresolved community incidents in the Musienene health zone in North Kivu; persistence of rumors about vaccine availability in the Hoho health zone (Rwampara health zone); in Haut-Uélé, uncontrolled parallel interventions by awareness actors and communication materials available only in French and Swahili, without translation into local languages.

1.8. Logistics

1.8.1. Main actions

In Ituri, the installed capacity reached 985 beds (496 for confirmed cases, 489 for suspected cases); delivery of 198 packages of PCI kits (425 kg) to the Mangala Health Zone and 11 packages of surveillance kits (60 kg) to the WHO office, preparation underway for a delivery to the Wamba Health Zone, and distribution of 515 liters of fuel; continuation of backfilling and paving work at the Nia-Nia CTE.

In North Kivu : receipt of IPC inputs consisting of masks (donation from Village Rich) and continuation of the development of the distribution plan for dignity kits received from UNFPA.

1.8.2. Challenges

Persistence of logistical needs to support the expansion of the response, including the completion of CTE extensions, the continued supply of essential inputs and PPE, and the availability of means of transport and evacuation.

1.9. Security

1.9.1. Main actions

- Securing the response activities continued in North Kivu with four public order maintenance actions carried out at 100%: security coverage of a Caritas Goma activity and the decontamination team in the AS of Butsili, and support for EDS teams for two burials of confirmed cases at the public cemetery and the family cemetery of Bundji.

1.9.2. Challenges

- Persistent insecurity and community resistance during dignified burials and secure, limiting access for response teams; insecurity reported in some villages opening rings in Ituri, and insufficient means of transport for security teams in North Kivu.

1.10. PSEA (Prevention of Sexual Exploitation and Abuse)

1.10.1. Main actions

- Continued awareness-raising activities on Prevention of Sexual Exploitation and Abuse (PSEA) in the various communities of the affected provinces.

1.10.2. The challenges

Coverage of awareness-raising activities and strengthening of mechanisms for the prevention, reporting and management of cases of sexual exploitation and abuse is still insufficient in all areas concerned by the response.

KEY MESSAGES

- Ebola virus transmission remains active, with 62 new cases and 30 deaths, including 14 fatalities Community and 16 intra-CTE cases were recorded in the last 24 hours, bringing the national total to 4,727 confirmed cases and 2,214 deaths. • A new health zone, Tshopo Health Zone, in Tshopo Province has been affected, bringing the number of affected provinces to 6, with 55 ZS.
- Ituri remains the epicenter of the epidemic, concentrating 85.0% of cumulative confirmed cases and 66.1% of new cases reported in the last 24 hours and a greater expansion (28 of the 36 health zones affected).
- Surveillance has performed well over the past 24 hours, with 94.5% of suspected cases investigated, a follow-up rate of 82.6% below the threshold of 85.0%.
- Strengthening priority interventions (surveillance, care, IPC, logistics and community engagement) remains essential to quickly interrupt transmission chains.

TIMELINE OF KEY EVENTS

- From April 24 to May 15, 2026, the epidemic progressed from the detection of the first suspected cases to the biological confirmation of the Ebola Bundibugyo virus, leading to the official declaration of the 17th Ebola Virus Disease epidemic in the Democratic Republic of Congo.
- Between May 17 and 18, 2026, national authorities, with the support of partners, strengthened the response by declaring a public health emergency of national and then continental scope, accompanied by the mobilization of coordination and support mechanisms.



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